

Code Billed: _____ Consult: Yes ☐ No ☐ If yes, <u>ALL</u> 3 must be documented (Request ☐ Report ☐ Recommendation ☐)							
Chief Complaint: _____ Bene Initials: _____ D.O.S. _____							
HISTORY/HISTORY	HPI (history of present illness) elements:			Brief 1-3 HPI elements Extended > 4 HPI elements or status of ≥ 3 chronic or inactive conditions			
	☐ Location Where is problem?	☐ Duration Onset of .symptoms to present.	☐ Modifying Factors What have you done to alleviate or worsen symptoms?				
	☐ Severity How bad on a scale 1/10	☐ Timing When/how often	☐ Associated Signs/Symptoms What else is bothering you?				
	☐ Quality Sharp/dull/ hot/dry	☐ Context What are you doing when sxs occurs?					
	ROS (Review of Systems)			None 1 ROS	Extended 2-9 ROS Complete ≥ 10 ROS or some systems + statement "all others negative"		
	☐ Constitutional ☐ Card/Vasc ☐ Musculo ☐ Psych ☐ "All Others Negative"	☐ Eyes ☐ Respiratory ☐ Integument ☐ Endo	☐ Ears, Nose Mouth, Throat ☐ GI ☐ GU ☐ Hem/Lymph				
	No PFSH is required: Subsequent Hospital and Subsequent Nursing Facility Care services require an interval history only.			Established/ Subsequent *E.D.	None	1 PFSH	2 PFSH
	☐ Past History (the pt's past experiences w/illnesses, operations, injuries, treatments, medications & allergies)						
	☐ Family History (review of medical events in the pt's family including diseases which are hereditary or put the pt at risk)			New or Initial	None	1-2 PFSH	3 PFSH
	☐ Social History (an age appropriate review of past and current activities)						
Circle the entry farthest to the right for each history area. To determine history level, draw a line down the column with the circle farthest to the left .				PROB. FOCUSED PF	EXP. PROB. FOCUSED EPF	DETAILED D	COMPRE- HENSIVE C
Important Note: Allow a comprehensive history if the physician is unable to obtain a history from the patient or other source . The record should describe the patient's condition or circumstance that precludes obtaining history. *99281-99285: No distinction is made between new & established patients in the E.D.							
EXAM	1997 Guidelines - General Multi System		1997 Guidelines - Single Organ System		Exam Level		
	1-5 elements identified by •		1-5 elements identified by •		PROBLEM FOCUSED (PF)		
	≥ 6 elements identified by •		≥ 6 elements identified by •		EXPANDED PF (EPF)		
	≥ 2 elements identified by • from any 6 areas/systems OR ≥12 elements identified by • from ≥2 areas/systems		≥ 12 elements identified by • EXCEPT ≥ 9 elements identified by • for eye & psych exams		DETAILED (D)		
	≥ 2 elements identified by • from 9 areas/systems		Document <u>all</u> elements in bolded outlined system boxes <u>and</u> ≥ 1 element in unbolded system boxes		COMPREHENSIVE (C)		
	Affected Body Areas (BA)		Organ Systems (OS)		1995 Guidelines		
☐ Head/Face ☐ Neck ☐ Abdomen ☐ Chest + breast / axillae ☐ Genital/groin/buttocks ☐ Back, include spine ☐ Extremity/(ies) L / R Upper L / R Lower	☐ Constitutional ☐ Eyes ☐ Ears, nose, mouth, throat ☐ Cardiovascular ☐ Respiratory ☐ GI ☐ GU ☐ Musculo	☐ Skin ☐ Neuro ☐ Psych ☐ Hem/Lymph/Immune	1 (BA) or (OS) (Limited exam of affected BA or OS)	2-7 (OS) and/or (BA) (Limited exam of affected BA or OS and other symptomatic or related OS(s))	2-7 (OS) and/or (BA) (Extended exam of affected BA(s) and other or related OS(s))	8 or more (OS) (A general multisystem exam or complete exam of a single organ system)	
			PF	EPF	D	C	

DECISION MAKING	A	Presenting Problems to the Treating Provider (# Diags Require Active Management or Affect Treatment Options)			B	Amount and/or Complexity of Data to be Reviewed Pts.		
			Points = Result					
		Self limited / minor (stable, improved or worse)	Max = 2	1			Review or order of clinical lab tests	1
		Est. problem (stable, improved)	Max = 2	1			Review or order of tests in the radiology section of CPT	1
		Est. problem (worsening)	Max = 2	2			Review or order of tests in the medicine section of CPT	1
		New problem (to Provider) (no add'l workup)	Max=1	3			Discussion of test results with performing physician	1
		New problem (to Provider) (additional workup)	Max=1	4			Decide to obtain old records or to obtain history from someone else	1
							Review & summarize old records <u>or</u> get Hx from someone <u>or</u> talk with other provider	2
		Bring total to Line A in Final Result for Complexity TOTAL					Independent visualization of image, tracing or specimen itself (not simply review of the paper copy report)	2
							Bring total to Line B in Final Result for Complexity TOTAL	
TIME	C	Risk of Complications / Morbidity / Mortality: Check off all that apply. The highest level of risk in any one column determines the <u>overall</u> risk.						
	Level	Presenting Problem(s)	Diagnostic Procedure(s) Ordered		Management Options Selected			
	MINIMAL	One self-limited or minor problem, e.g., cold, insect bite, tinea corporis	- Laboratory tests requiring venipuncture - Chest x-rays, KOH prep or EKG/EEG - Urinalysis or Ultrasound e.g., echo - Potassium Dydroxide prep etc.		- Rest - Gargles - Elastic bandages - Superficial dressings			
	LOW	- Two or more self-limited or minor problems - One stable chronic illness e.g., well controlled hypertension, non-insulin dependent diabetes, cataract, BPH - Acute uncomplicated illness or injury e.g., cystitis, allergic rhinitis, simple sprain	- Physiologic test not under stress e.g., pulm. function tests - Non-cardiovascular imaging studies with contrast e.g., barium enema - Superficial needle biopsies or Skin biopsies - Clinical laboratory tests requiring arterial puncture		- Over the counter drugs - Minor surgery with no identified risk factors - Physical therapy - Occupational therapy - IV fluids without additives			
	MODERATE	- One or more chronic illnesses with mild exacerbation, progression or side effects of treatment - Two or more stable chronic illnesses - Undiagnosed new problem with uncertain prognosis e.g., lump in breast - Acute illness with systemic symptoms e.g., pyelonephritis, pneumonia, colitis - Acute complicated injury e.g., head injury with brief loss of consciousness	- Physiologic test under stress e.g., cardiac stress test, fetal contraction stress test - Diagnostic endoscopies with no identified risk factors - Deep needle or incisional biopsy - Cardiovascular imaging studies with contrast and no identified risk factors e.g., arteriogram, cardiac cath - Obtain fluid from body cavity e.g., lumbar puncture, thoracentesis, culdocentesis		- Minor surgery with identified risk factors - Elective major surgery (open percutaneous or endoscopic) with no identified risk factors - Prescription drug management - Therapeutic nuclear medicine - IV fluids with additives - Closed treatment of fracture or dislocation without manipulation			
	HIGH	- One or more chronic illnesses with severe exacerbation, progression or side effects of treatment - Acute or chronic illnesses or injuries that may pose a threat to life or bodily function e.g., multiple trauma, acute MI, pulmonary embolus, severe respiratory distress, progressive severe rheumatoid arthritis, psychiatric illness w/potential threat to self or others, peritonitis, acute renal failure - An abrupt change in neurological status e.g., seizure, TIA, weakness, sensory loss	- Cardiovascular imaging studies with contrast with identified risk factors - Cardiac electrophysiological tests - Diagnostic endoscopies with identified risk factors - Discography		- Elective major surgery (open, percutaneous or endoscopic) with identified risk factor - Emergency major surgery (open, percutaneous or endoscopic) - Parenteral controlled substances - Drug therapy requiring intensive monitoring for toxicity - Decision not to resuscitate or de-escalate care because of poor prognosis			
	A	Circle the Total number in section A	≤ 1 Minimal	2 Limited	3 Multiple	≥ 4 Extensive		
	B	Circle the Total number in section B	≤ 1 Minimal or None	2 Limited	3 Multiple	≥ 4 Extensive		
	C	Circle the Level in section C	Minimal	Low	Moderate	High		
		Complexity Level of Medical Decision Making (Mdm)	STRAIGHTFORWARD SF	LOW L	MODERATE M	HIGH H		
	Draw a line down the column with 2 or 3 circles and circle decision making level OR Draw a line down the column with the center circle = level of Mdm							
	If the physician documents total time and suggests that counseling or coordinating care dominates (greater than 50%) the encounter, time may determine level of service. Documentation may refer to: prognosis, differential diagnosis, risks, benefits of treatment, instructions, compliance, and/or risk reduction.					If both answers are "yes," you may select the level based on time.		
	Does documentation reveal total time? Time: Face-to-face outpatient setting Unit/floor in inpatient setting	☐ Yes	☐ No					
	Does documentation describe the content of counseling or coordinating care?	☐ Yes	☐ No					

PLEASE NOTE: Time factors are indicated by CPT code followed by –xx (example: 99201-10 indicates 10 minutes)

Directions: Transfer the history, exam and medical decision making results to the correct chart below & follow the instructions for that Code family

New Office/Outpatient Visits & Office/Inpatient Consultations						Established Patient Office/Outpatient Visits				
Level	Draw a line down the column which has a key component identified which is the farthest to the left (leveled by the lowest)					If a column has 2 or 3 circles, draw a line down the column and circle the code OR draw a line down the column with the center circle and circle the code				
HX	PF	EPF	D	C	C	Minimal problem that may not require presence of MD/DO	PF	EPF	D	C
EX	PF	EPF	D	C	C		PF	EPF	D	C
MDM	SF	SF	L	M	H		SF	L	M	H
CPT Code	99201-10 99241-15 99251-20	99202-20 99242-30 99252-40	99203-30 99243-40 99253-55	99204-45 99244-60 99254-80	99205-60 99245-80 99255-110	99211-5	99212-10	99213-15	99214-25	99215-40

Initial Hosp. Visits & Observation Care				Subsequent Hosp.		
Level	Draw a line down the column which has a key component identified which is the farthest to the left (leveled by the lowest) These are <u>PER DAY CODES</u>			If a column has 2 or 3 circles, draw a line down the column and circle the code OR draw a line down the column with the center circle and circle the code This is a <u>PER DAY CODE</u>		
HX	D or C	C	C	PF interval	EPF interval	D interval
EX	D or C	C	C	PF	EPF	D
MDM	SF/L	M	H	SF/L	M	H
CPT Code	99221-30 99218 99234	99222-50 99219 99235	99223-70 99220 99236	99231-15	99232-25	99233-35

EMERGENCY CARE SERVICES					
Draw a line down the column which has a key component identified which is the farthest to the left (leveled by the lowest)					
HX	PF	EPF	EPF	D	C
EX	PF	EPF	EPF	D	C
MDM	SF	L	M	M	H
CPT Code	99281	99282	99283	99284	99285

Abbreviation Legend:

CC = Chief Complaint
HX = History
PF = Problem Focused
SF = Straightforward

ROS = Review of System
EX = Exam
EPF = Expanded Problem Focused
L = Low

PFSH = (Past, Family, Social) History
Mdm = Medical Decision Making
D = Detailed
M = Moderate
C = Comprehensive
H = High

Additional Comments: _____

Directions: Transfer history, exam and medical decision making results to appropriate chart below and follow the specific instructions for chart.

The Nursing Facility Care Codes are PER DAY CODES, time factors effective 2008

	Initial Nursing Facility Care			Subsequent Nursing Facility Care			
Level	Draw a line down the column which has a key component identified which is the farthest to the left (leveled by the lowest)			If a column has 2 or 3 circles, draw a line down the column and circle the code OR draw a line down the column with the center circle and circle the code			
HX	D	C	C	PF interval	EPF interval	D interval	C interval
EX	D	C	C	PF	EPF	D	C
MDM	L	M	H	SF	L	M	H
CPT Code	99304-25	99305-35	99306-45	99307-10	99308-15	99309-25	99310-35

	New Patient Home/Domiciliary/Custodial/Rest Home Etc.					Established Home/Domiciliary/Custodial/Rest Home Etc.			
	Draw a line down the column which has a key component identified which is the farthest to the left (leveled by the lowest).					If a column has 2 or 3 circles, draw a line down the column and circle the code OR draw a line down the column with the center circle and circle the code			
HX	PF	EPF	D	C	C	PF interval	EPF interval	D interval	C interval
EX	PF	EPF	D	C	C	PF	EPF	D	C
MDM	SF	SF	L	M	H	SF	L	M	M to H
CPT Code	99341-20 99324-20	99342-30 99325-30	99343-45 99326-45	99344-60 99327-60	99345-75 99328-75	99347-15 99334-15	99348-25 99335-25	99349-40 99336-40	99350-60 99337-60

Abbreviation Legend: CC = Chief Complaint HX = History PF = Problem Focused SF = Straightforward	ROS = Review of System EX = Exam EPF = Expanded Problem Focused L = Low	PFSH = (Past, Family, Social) History Mdm = Medical Decision Making D = Detailed M = Moderate C = Comprehensive H = High
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Additional Comments: _____
